

RIVERSIDE GENERAL HOSPITAL

123 Medical Drive, Springfield IL 62701 | Cardiology & Internal Medicine

DISCHARGE MEDICATION SUMMARY

Patient: Kent Romaguera DOB: January 9, 1940 Date: May 30, 2026
MRN: 25127-BD5-F9AA Gender: Male Age: 86 yrs Unit: Cardiology Ward 3B
Attending: Dr. Maria Chen MD Admission: 2026-05-20 Discharge: 2026-05-28

PRIMARY DIAGNOSES

- Atrial fibrillation with rapid ventricular response (I48.91)
- Hypertensive heart disease with heart failure, stage II (I11.0)
- Coronary artery disease, native vessel, without angina (I25.10)
- Type 2 diabetes mellitus without complications (E11.9)
- Chronic kidney disease, stage 3a (N18.31)

MEDICATIONS AT DISCHARGE

* Patient prescribed concurrent warfarin + aspirin. See pharmacist alert below.

Medication	Dose/Route	Indication	Frequency
Warfarin sodium 5 mg	5 mg oral	Atrial fibrillation - stroke	Daily (adj.)
Aspirin 81 mg	81 mg oral	CAD - antiplatelet	Once daily
Metoprolol succinate ER	50 mg oral	HF / rate control	Once daily
Lisinopril	10 mg oral	HF / hypertension	Once daily
Furosemide	40 mg oral	HF - volume overload	AM daily
Atorvastatin	40 mg oral	CAD / dyslipidemia	PM daily
Metformin	500 mg oral	Type 2 diabetes	BID meals
Potassium chloride	20 mEq oral	Hypokalemia prevention	Once daily

CLINICAL PHARMACIST NOTE - DRUG INTERACTION ALERT

ALERT: Warfarin + Aspirin concurrent use detected

This patient is currently prescribed both warfarin (anticoagulant) and aspirin 81 mg (antiplatelet). In adults aged 65 and older, this combination significantly increases the risk of major bleeding events, including gastrointestinal hemorrhage and intracranial bleeding.

Evidence: AGS 2023 Beers Criteria (rule beers-2023-interaction-01) lists warfarin + aspirin as HIGH evidence, STRONG recommendation to avoid in older adults. The absolute risk of major bleeding approximately doubles with dual therapy.

Action required: Confirm dual therapy is clinically indicated (e.g., recent ACS or coronary stent). If both agents are necessary, co-prescribe a proton pump inhibitor (e.g., pantoprazole 40 mg daily) for gastroprotection. Physician acknowledgement required within 48 hours. INR target: 2.0-3.0. Next INR check: 3 days post-discharge.

RECENT LABORATORY VALUES (2026-05-27)

INR	2.4	Therapeutic (target 2.0-3.0)
Creatinine	1.6 mg/dL	Elevated - consistent with CKD stage 3a
eGFR	42 mL/min/1.73m2	Reduced - monitor renally-cleared drugs
Potassium	3.8 mEq/L	Normal
HbA1c	7.2%	Acceptable glycemic control
BNP	380 pg/mL	Elevated - HF, improving

SYNTHETIC DATA - NOT A REAL PATIENT. Generated for Umraa project. Source: Synthea (Kent Romaguera).